

1. Administrative & Study Identification

Full Study Title: RACE-2L: Real-World Assessment of Clinical Practice and Outcomes in Non-squamous Non-Small Cell Lung Cancer After Failure of Platinum-based Chemotherapy in Brazil **Short Title/Acronym:** RACE-2L **Protocol Number:** D133HR00041 **NCT Number:** NCT06491862 **Sponsor Name:** AstraZeneca **Investigational Product:** N/A (Observational review of real-world systemic treatments) **Phase of Development:** N/A (Observational / Retrospective) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate and describe the real-world clinical practice, treatment patterns, and outcomes of patients in routine practice settings who have been diagnosed with advanced/metastatic (a/m) non-squamous NSCLC with or without actionable genomic alterations (AGA) and received at least 1 line of systemic treatment after failing platinum-based chemotherapy (PTC). **Secondary Objectives:** To evaluate treatment outcomes (such as real-world survival endpoints) and conduct subgroup analyses on pre-defined NSCLC subtypes (EGFR mutant, ALK translocation, ROS1 translocation, MET1 mutant, HER-2 mutant, BRAF mutant, pan-wild-type PD-L1 >50%).

2.2 Background & Rationale To address the knowledge gap regarding real-world treatment sequencing and clinical outcomes for advanced/metastatic non-squamous NSCLC patients after the failure of platinum-based chemotherapy. Generating real-world evidence from clinical settings in Brazil will help better understand routine practice and treatment efficacy outside the scope of controlled clinical trials.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Observational (Retrospective, noninterventional medical record review) **Control Type:** Single-arm / Historical Cohort **Blinding:** Open-label (N/A for retrospective chart review) **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target \$N\$:** Sequential adult patients meeting eligibility (dependent on medical records availability) **Number of Sites:** Routine clinical practice settings across Brazil **Estimated Study Start:** 25-Jul-2024 **Estimated Primary Completion:** 30-Jun-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult patients (≥ 18 years) diagnosed with stage IIIB/IIIC/IV advanced/metastatic non-squamous NSCLC with or without actionable genomic alterations (AGA). 2. Received at least 1 line of systemic treatment after failing platinum-based chemotherapy (PTC) with a start date between January 2017 and December 2024. (*Note: For patients without AGA, must have failed PTC and immunotherapy. For patients with AGA, must have failed PTC and at least 1 line of target approved therapy*). 3. The study index date must be definable (defined as the first dose of any agent after PTC).

4.2 Exclusion Criteria 1. Evidence of other primary malignancies (apart from basal cell carcinoma and melanoma) within 1 year prior to the index date. 2. Participated in an investigational clinical trial for the treatment of any cancer, including NSCLC, or any early access program from 2018 onward.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: N/A (Retrospective observation of real-world systemic treatments, including targeted therapy, immunotherapy, or subsequent chemotherapy as prescribed by the treating physicians). **Route of Administration:** Varies (Oral, IV, etc.) based on standard clinical practice for the respective agents. **Duration of Treatment:** Extracted retrospectively from the index date until death, last medical record entry, or date of data extraction.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care treatments and concomitant supportive medications administered in routine real-world practice. **Prohibited:** Use of any investigational drug or participation in an interventional clinical trial from 2018 onward precludes patient inclusion.

6. Study Timeline & Visit Schedule

(*Note: As a retrospective chart review, windows apply to data collection rather than prospective visits*) | Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Index Date** | Day 0 | First dose of any active agent after failing Platinum-Based Chemotherapy (PTC). | | **Observation Period** | Jan 2017 – Dec 2024 | Retrospective extraction of treatment sequences, disease progression, and mutation status (EGFR, ALK, ROS1, etc.). | | **End of Follow-Up** | Varies | Date of death, last medical record entry, or data extraction limit. |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Real-world clinical practice and treatment patterns (treatment sequencing) in a/m non-squamous NSCLC patients after failure of PTC. **Measurement Method:** Retrospective electronic medical record (EMR) / chart abstraction.

7.2 Secondary & Exploratory Endpoints Real-world clinical outcomes including Real-world Overall Survival (rwOS) and Real-world Progression-Free Survival (rwPFS) or Time to Next Treatment (TTNT). Subgroup analysis of outcomes based on pre-defined NSCLC subtypes (EGFR mutant, ALK translocation, ROS1 translocation, MET1 mutant, HER-2 mutant, BRAF mutant, pan-wild-type PD-L1 >50%).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Retrospective extraction of documented treatment-related adverse events from medical records. **Serious Adverse Events (SAEs):** Captured based on hospitalization records or severity noted in the routine clinical documentation. **Data Monitoring Committee (DMC):** N/A (Standard for non-interventional, retrospective data reviews).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Full Analysis Set consisting of all sequential patients meeting the strict inclusion and exclusion criteria based on medical records. **Sample Size Justification:** Sample size will rely on a convenience sample of all available and eligible sequential patients at participating sites from January 2017 to December 2024. **Handling of Missing Data:** Standard epidemiological handling of missing real-world data (e.g., right-censoring for time-to-event survival outcomes).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local ethical frameworks governing retrospective patient data handling and anonymization in Brazil. **Monitoring Plan:** Data abstraction quality checks and source data verification (SDV) on a predefined percentage of entered ECRFs. **Audit Trail:** Validated, secure electronic database with audit trails utilized for all abstracted medical records.

11. Study Identifiers & Governance

Primary ID: D133HR00041 **Registry Number:** NCT06491862 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** RACE-2L **Official Regulatory Title:** RACE-2L: Real-World Assessment of Clinical Practice and Outcomes in Non-squamous Non-Small Cell Lung Cancer After Failure of Platinum-based Chemotherapy in Brazil

12. Clinical Framework (NSCLC Specific)

Primary Condition: Advanced/Metastatic Non-squamous Non-Small Cell Lung Cancer (NSCLC) **Diagnosis Threshold:** Failure of Platinum-based Chemotherapy (PTC) **Medical Methodology:** Real-world routine clinical practice (Targeted Therapy, Immunotherapy, Chemotherapy) **Optimization Target:** Identifying optimal treatment sequences and improving survival outcomes for specific AGA vs. non-AGA profiles

13. Study Procedures & Methodology

Management Pathway: Routine standard of care clinical pathway for a/m NSCLC in Brazil. **Education Protocol (HCP):** N/A (Retrospective cohort study) **Education Protocol (Patient):** N/A (Retrospective cohort study) **Quality Audit Mechanism:** Electronic medical record screening and periodic sponsor source data verification for extraction accuracy.

14. Observation & Follow-Up Schedule

Total Duration: Retrospective follow-up spans from the individual patient index dates (between Jan 2017 and Dec 2024) until study termination data limits. **Onsite Visit Cadence:** N/A (Chart extraction framework) **Remote Monitoring Frequency:** Continuous data entry and validation during the extraction period. **Checklist Review Interval:** Scheduled data cleaning and quality review milestones by the clinical data management team.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				